Introduction

The stethoscope made audible what was once imperceptible; similarly, the psychiatric diagnosis lens attempts to make comprehensible the intangible contours of the human mind. But is this lens capturing not only disorder but also ordinary life experience? This debate has recently entered public discourse with Health Secretary Wes Streeting arguing that normal distress can be medicalised, causing too many people to be “written off” (Nevett, 2025).

To evaluate this claim of overdiagnosis, it is first necessary to establish that diagnosis rates are, in fact, increasing. According to research commissioned by NHS England, rates of common mental health conditions such as depression and anxiety have increased in recent decades. In 2007, 17.6% of adults were identified with such conditions; in 2014, 18.9%; by 2025, that rate had grown to 22.6% (National Centre for Social Research, 2025). This trend is also reflected globally, with nearly one in seven people worldwide living with a mental condition, such as anxiety and depressive disorders (World Health Organization, 2025). The question, therefore, is not whether diagnosis rates are rising, but why.

Distinguishing ordinary suffering from mental disorder is central to evaluating claims of overdiagnosis. Ordinary suffering refers to emotional distress arising from life events. Experiences such as grief, anxiety, and bereavement may be intense but not necessarily indicative of disorder (American Psychiatric Association, 2013). Conversely, mental illness refers to a clinically significant disturbance in cognition, emotion, or behavior that causes substantial distress or impairment in education, relationships, or work (World Health Organization, 2025).

What, then, is causing this increase in mental illness diagnoses? Some suggest overdiagnosis, others claim improved recognition, while yet others cite real rises in disorders. This paper argues that all three perspectives contribute to rising diagnoses.

Overdiagnosis and better recognition are therefore not mutually exclusive, but coexist alongside genuine increases in mental illness.

Overdiagnosis

Streeting and other politicians seeking to reduce government costs are not the only ones who believe mental illness is being overdiagnosed. As Allen Frances, former chair of the DSM-IV task force, warned, “The elastic boundaries of psychiatry have been steadily expanding, because there is no bright line separating the worried well from the mildly mentally disordered” (Frances, 2013). Moreover, psychiatrist Thomas Szasz argued that supposed mental illnesses are not diseases in the conventional medical sense, but rather problems in living that have been medicalised. Contending that mental disorders were merely “a myth, whose function it is to disguise and thus render more palatable the bitter pill of moral conflicts in human relations” (Szasz, 1960), he viewed diagnoses as social classifications of behaviours that deviate from cultural norms. Although Szasz’s critique should not deny severe mental illness, it accentuates how psychiatric categories are shaped by cultural and societal norms.

Szasz’s position is more radical than Frances’s, but both caution against allowing diagnostic categories to expand beyond clear dysfunction. In this sense, rising diagnostic rates may partly reflect an increasingly expansive psychiatric framework rather than a proportional increase in genuine mental illness. How, then, have cultural and societal norms shifted to encourage overdiagnosis?

One case for overdiagnosis lies in structural and cultural incentives. In the current landscape of pharmacotherapy, pharmaceutical marketing and healthcare systems encourage medical diagnosis and the prescription of medication. For example, direct-to-consumer advertising (particularly in the US) encourages patients to interpret everyday struggles as symptoms. This institutional tendency can be observed through prescription patterns: only about 5% of the population has a severe mental disorder, yet

roughly 20% take a psychotropic medication each year, and 80% of these prescriptions are written by primary care physicians rather than psychiatrists (Frances, 2013).

Although pharmaceutical innovation has undoubtedly helped many people, the commercial imperative to expand drug use beyond clinical necessity has normalized medication as the default response even when non-pharmacological approaches such as psychotherapy may be more appropriate (Moynihan et al., 2002). Beyond corporate motivations, diagnoses can also function as gateways to services. Insurance reimbursement, for instance, often requires a formal diagnosis; therefore, without a diagnosis, patients may struggle to access treatments. Clinicians may therefore feel pressured to label patients in order to submit billing codes.

This system that rewards intervention has altered the cultural expectations of medicine, making societies now less tolerant of uncertainty, sadness, and emotional pain. As discomfort has become a symptom to be treated rather than an inconvenience to be endured, medicine has acquired immense cultural authority. This establishes a self-perpetuating cycle, with common distress increasingly interpreted through a medical lens, prompting more people to seek diagnosis and treatment. The more society relies on psychiatric categories to explain distress, the more distress is understood as psychiatric in nature. This lowering of the clinical threshold paired with the expanding number of psychiatric categories has resulted in the pathologisation of normal human responses to environmental stress, further fueling overdiagnosis (Frances, 2013; Thombs et al., 2019). This concern is echoed by British psychiatrist Joanna Moncrieff, who argues that many mental health diagnoses function less as objective medical explanations than as labels shaped by “commercial, professional and political interests” (Moncrieff & McPherson, 2025). From this perspective, rising diagnostic rates may reflect the growing medicalisation of social and emotional distress rather than a proportional increase in mental health illnesses.

Mental illness diagnoses can appeal not only to those who benefit economically but also to fundamental aspects of human psychology. On an individual level, diagnostic labels

satisfy profound psychological and social needs by providing explanation, identity, and belonging (Corden et al., 2021). For example, many adults diagnosed with autism describe the label as a means of making sense of lifelong feelings of difference, enabling them to reinterpret struggles and connect with others who share similar experiences (Corden et al., 2021).

This appeal is particularly pronounced during adolescence. In this context, the rise of social media and accessible psychological information has accelerated self-diagnosis of mental illness. Through processes such as confirmation bias, adolescents may encounter relatable symptom lists and thus misdiagnose themselves with clinical labels.

However, while such labels provide validation and clarity, they can become central to personal identity and thus risk expanding the threshold for what is considered pathological. Consequently, ordinary experiences may be reinterpreted as clinical symptoms: forgetfulness as ADHD, social awkwardness as autism, and normal mood fluctuations as bipolar disorder, thus contributing to the rise of overdiagnosis.

Improved Recognition

Rising diagnoses, however, cannot fully be explained by overdiagnosis. Other contributing factors include improved psychological recognition of conditions along with greater awareness and acceptance of mental illness. Refined assessment instruments, longitudinal research, and enhanced comprehension of how symptoms vary by age, gender, and socio-cultural background have made diagnosis more accurate and inclusive. Conditions such as autism and ADHD in girls, depression in men, or anxiety disorders in children are now more likely to be recognised.

The rise of social media and online psychological information has not only contributed to increases in misguided self diagnosis, but also reduced stigma and spread awareness of mental illnesses (Zhang et al., 2026). Online communities have enabled individuals

from historically misunderstood groups to articulate experiences that may previously have gone unrecognised.

Women, ethnic minorities, and those whose symptoms diverge from traditional diagnostic stereotypes have often been underdiagnosed, partially due to psychiatric research and diagnostic criteria based on narrower, less representative populations (Singh & Wendt, 2024). Digital platforms have challenged these limitations, amplifying diverse experiences and perspectives, and broadening public understanding of the manifestation of mental illnesses across different groups. This increased willingness to seek support is reflected in treatment patterns: 47.7% of adults with symptoms of common mental health conditions reported receiving treatment in 2025, compared to 39.4% in 2014, and just 24.4% in 2007 (National Centre for Social Research, 2025). Hence, rising diagnoses may reflect a correction of historical shortcomings in psychiatric practice combined with a decrease in social stigma associated with mental illness.

Genuine Increases in Mental Illness

Overdiagnosis and improved awareness have contributed to increased rates of diagnosis, but what if people now genuinely experience more mental illness than previous generations did? Research suggests that economic pressures, social media, and earlier puberty influence genuine mental illness.

Modern life has created new social and economic pressures that contribute to worsening mental health. Economic insecurity, academic competition, and widening inequality have exacerbated the psychological burdens of everyday life (American Psychological Association, 2023). The COVID-19 pandemic further intensified these pressures through social isolation, educational disruption, and uncertainty about the future. Therefore, PTSD has been more commonly reported among younger adults and those experiencing socioeconomic disadvantage (National Centre for Social Research,

2025), reflecting a genuine rise in psychological distress rather than merely increased awareness.

Social media's multifaceted psychological impacts may also contribute to genuine increases in mental illness. On one hand, excessive reliance on social media reduces face-to-face interaction, contributing to social isolation and loneliness (Fassi et al., 2024). Furthermore, social comparison diminishes self-esteem and heightens feelings of inadequacy, particularly through constant exposure to idealised lifestyles and appearances, while cyberbullying and algorithmic reinforcement can aggravate distress (Anichitoae et al., 2024; Sharma & Sharma, 2026). For adolescents already undergoing significant hormonal, cognitive, and social changes, these digital environments amplify existing psychological vulnerabilities.

Indeed, rates of mental illness in adolescents are not only impacted by the spread of social media but also by the earlier onset of puberty itself. Puberty is a key stage in biological development, during which adolescents experience profound physical, cognitive, and emotional changes as they transition from childhood to adulthood. While these changes and their psychological implications have long been studied, the effects on mental health of the earlier onset of puberty remain a largely unstudied phenomenon. Since the 1970s, studies have found that puberty is occurring earlier, particularly in girls. The age of breast development, for instance, has advanced by nearly three months per decade since 1977 (Eckert-Lind et al., 2020). Similarly, the average age of menarche has fallen from 12.5 years among those born in 1950-1969 to 11.9 years among those born in 2000-2005 (Wang et al., 2024). In contrast, diagnosed conditions among 12-17 year-olds rose from 15.0% in 2016 to 20.3% in 2023 (Sappenfield et al., 2024).

This early onset of puberty has created a mismatch between physical maturity and emotional readiness that increases vulnerability to anxiety, depression, and social pressures (Mendle et al., 2007). Because brain regions involved in emotion regulation are still developing during puberty, hormonal fluctuations can heighten emotional

sensitivity (Ahmed et al., 2015). Repercussions include increased vulnerability to depression, higher rates of anxiety disorders, and greater susceptibility to body image concerns (Graber, 2013; Vannucci & Ohannessian, 2018).

Furthermore, these vulnerabilities are compounded by social pressures. Early-maturing adolescents, for instance, may face unwanted sexual attention, social alienation, identity confusion, and pressures to behave with maturity they have not fully developed (Mendle et al., 2007). Adolescents may therefore find themselves navigating complex issues before developing the psychological resources to cope, which contributes to the rising rates of valid mental illness diagnoses.

Conclusion

Rising rates of mental illness do not reflect a single causative phenomenon, but the convergence of several forces. They are partly the result of a genuine increase in psychological distress, partly the product of improved recognition and more inclusive diagnostic practices, and partly a consequence of the expanding medicalisation of normal human suffering. Principles traditionally used in clinical diagnosis, such as functional impairment, persistence over time, proportionality to context, and clinical utility, provide a useful framework for interpreting these trends. Where symptoms are enduring, disabling, and responsive to treatment, rising rates are more likely to reflect genuine disorder. Where they are transient, contextually understandable, and minimally impairing, they may instead indicate the medicalisation of ordinary human experiences. The most productive approach, therefore, is not to ask whether rising rates are “real” or “artificial,” but to determine under what conditions each interpretation is justified. Focusing the psychiatric diagnosis lens on this distinction is crucial in comprehending the contours of the human mind. Dismissing increases in diagnoses risks neglecting genuine distress, while treating all suffering as pathological risks eroding the boundary between disorder and normal human experience.

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